June 16, 2025

Ms. Stephanie Carlton
Deputy Administrator
Centers for Medicare & Medicaid Services
Attention: CMS–0042–NC
P.O. Box 8013
Baltimore, MD 21244–8013

Dear Deputy Administrator Carlton:

RE: Request for Information: Health Technology Ecosystem (CMS–0042–NC)

On behalf of WyDus, LLC, thank you for the opportunity to comment on the Request for Information titled *Health Technology Ecosystem* issued by the Centers for Medicare & Medicaid Services (CMS), the Assistant Secretary for Technology Policy (ASTP), and the Office of the National Coordinator for Health Information Technology (ONC).

WyDus, LLC is a consulting firm supporting digital health, diagnostic, and medtech innovators navigating reimbursement and market access in the U.S. healthcare system. These comments also reflect my personal perspective as a cancer survivor who has directly experienced the gaps in data access and care coordination highlighted by this RFI.

Summary of Recommendations

Improve Access to Second Opinions in Cancer Care
Launch a CMS demonstration program to support the development of a unified digital platform that enables patients to easily obtain second opinions. To encourage provider participation, the program should include reimbursement for digital pathology services and multidisciplinary tumor board reviews. The demonstration should assess the impact on diagnostic accuracy, treatment planning, rural access, and patient outcomes.

Increase Reimbursement Predictability for Digital Health Tools

- Establish an NTAP-equivalent program under the OPPS to support digital tools not currently eligible for TPT or new technology APCs.
- Create a structured and timely payment pathway for new digital health services delivered in physician office settings.
- Extend the NTAP period for digital health technologies to allow sufficient time for evidence development and support broader market adoption.

PC–2 and PC–4: Patient Access, Functionality, and Unmet Needs in Digital Tools

CMS asked whether patients have easy access to their health data, and what features are missing from existing digital tools. One major gap is the absence of a digital platform or app that facilitates timely second opinions for cancer diagnosis and treatment planning. A 2018 study in *Annals of Surgical Oncology* found that 43% of patients had their diagnosis or treatment plan changed after a second review by a multidisciplinary tumor board.[1] While most academic centers offer second opinion services, there is no unified platform that enables patients to identify the right specialists and transfer clinical data efficiently in a single portal.

Accessing a second opinion typically requires patients to gather and share imaging, biopsy slides, lab, and genomic test data, which can be overwhelming and lead to concerns about delaying treatment. Sharing pathology data is especially cumbersome. Although the FDA approved whole-slide imaging for primary diagnosis in 2017, pathology slides are still commonly exchanged by physically shipping tissue blocks.

A streamlined digital solution is particularly critical to reducing disparities in cancer care between urban and rural populations. Second reviews can help rural patients access expert diagnostic input while allowing them to receive ongoing treatment closer to home. Chemotherapy regimens can span many months; traveling to tertiary centers is not feasible for many patients.

CMS has long recognized the value of second opinions and Medicare currently covers these services. A Medicare demonstration project in the 1980s found that few beneficiaries used second opinion services, not due to lack of value, but due to non-monetary barriers such as travel time, difficulty locating specialists, and challenges in transferring medical records.[2] Today's digital infrastructure can remove those barriers—with the support of right platforms and reimbursement incentives.

To support broader adoption, we recommend CMS to launch a demonstration project that:

- Supports the development of a unified digital platform to facilitate second opinions for Medicare beneficiaries;
- Covers digital pathology services;
- Reimburses participation in multidisciplinary tumor board reviews;
- Assesses the impact on diagnosis accuracy, treatment planning, patient outcomes and rural care.

[1] Garcia D, Spruill LS, Irshad A, Wood J, Kepecs D, Klauber-DeMore N. The Value of a Second Opinion for Breast Cancer Patients Referred to a National Cancer Institute (NCI)-Designated Cancer Center with a Multidisciplinary Breast Tumor Board. Ann Surg Oncol. 2018 Oct;25(10):2953-2957. doi: 10.1245/s10434-018-6599-y. Epub 2018 Jul 3. PMID: 29971672; PMCID: PMC6132422.
[2] Friedlob AS. Medicare Second Surgical Opinion Programs: The Effect of Waiving Cost-Sharing. Health Care Financing Review: Vol 4, No. 1, 1982. https://www.cms.gov/Research-Statistics-Data-and-Systems/Research/HealthCareFinancingReview/Downloads/CMS1190360DL.pdf

TD–1: Stimulating Developer Interest in Digital Health Solutions for Medicare Beneficiaries

CMS asked what steps the agency can take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers. One of the most important actions CMS can take is to strengthen reimbursement predictability for digital health technologies. Investors and developers frequently cite unclear or delayed payment mechanisms as major barriers to innovation.

While Medicare currently offers several payment pathways for new technologies, including the New Technology Add-on Payment (NTAP) under the Inpatient Prospective Payment System (IPPS), the New Technology Ambulatory Payment Classification (New Technology APC), and Transitional Pass-Through (TPT) payments under the Outpatient Prospective Payment System (OPPS), these mechanisms lack consistency across care settings. Additionally, existing programs could be enhanced to better stimulate investment and innovation.

1. Improve Policy Consistency Across Payment Systems

CMS has established various payment policies to support adoption of new technologies, including:

- NTAP: Provides additional reimbursement to hospitals using eligible new technologies beyond the standard IPPS payment during a transitional period.
- New Technology APC: A special APC category under OPPS for innovative services or procedures that do not fit existing APC classifications.
- TPT: Offers temporary additional payment under OPPS for eligible new devices that are surgically implanted or inserted.
- OPPS Ratesetting Process for New Codes: Allows for quarterly review and temporary payment assignment for new Category III CPT codes, with final rates established through annual OPPS rulemaking process.

However, these policies operate in silos and are not aligned across care settings. Many digital health products are deployed across inpatient, outpatient, and physician office settings. The same service may be reimbursed in one setting but not in another. To stimulate digital health innovation, CMS should establish more consistent policies and a coordinated application process. We specifically recommend:

1.1. Create an NTAP-Equivalent Program Under OPPS

Currently, OPPS supports new technologies via new technology APC and TPT. However, TPT is limited to devices that come in contact with human tissue and are surgically implanted or inserted, and new technology APCs apply only to complete procedures. There are currently no opportunities for additional payment for new ancillary technologies that assist, rather than define, a procedure.

For example, Viz LVO, an AI-enabled software tool used in emergency departments (ED) to assist with stroke triage, received NTAP status in 2020. However, if a patient is not admitted as an inpatient and is instead discharged after an ED visit or after an outpatient observation stay, hospitals have no opportunity to receive additional payment under OPPS for using this technology.

As more care shift from inpatient to outpatient setting, OPPS should offer parity with IPPS by establishing an NTAP-equivalent pathway to support the adoption of innovative technologies across all care settings.

1.2. Establish a Timely Ratesetting Mechanism for Digital Health Tools Used in Physician Office

Unlike OPPS, the PFS lacks a structured process for temporarily assigning payment to new Category III CPT codes. Instead, most codes are priced by Medicare Administrative Contractors (MACs), resulting in inconsistent rates across jurisdictions and care settings. CMS should consider two possible solutions:

- Establish a timely and transparent process for assigning temporary payments to new Category III CPT codes under the PFS; or
- Expand the OPPS infrastructure to support reimbursement of digital health tools used in office-based settings, especially those without a professional component.

2. Strengthen Existing Programs to Support Digital Health Innovation

CMS should consider extending the NTAP transitional period for digital health tools. Currently, NTAP status is limited to two years, which is based on the assumption that sufficient cost data can be collected and incorporated into IPPS rate-setting in this time frame. This works well for high-cost technologies used within a narrow set of Diagnosis Related Groups (DRGs), such as CAR-T therapies.

However, digital health tools are often used across multiple DRGs. Because DRG weights are calculated based on relative costs, the cost impact of digital health solutions used broadly across DRGs is diluted or even canceled out in rate setting. As a result, there are no financial incentives to adopt new digital health tools under a bundled payment system like IPPS, once NTAP support expires, even when those tools improve clinical care.

We recommend that CMS extend the NTAP period for digital health technologies to allow sufficient time for successful product launch and for developers to generate the clinical and economic evidence needed to support long-term market access. Digital health tools often need to be integrated into hospital workflow, and the hospital procurement and implementation process can consume much of the current two-year NTAP window.

Extending the NTAP period is a fiscal responsible strategy, because

- Early adoption volumes are generally modest;

- Not all cases utilizing eligible technologies qualify for NTAP, which applies only when costs of an inpatient stay exceed the DRG payment;
- Even when NTAP is triggered, the payment amount may fall below the maximum allowed due to the calculation methodology;
- NTAP applications require evidence of clinical improvement, and many qualifying technologies have received FDA Breakthrough Device designation. Improved clinical outcomes often lead to shorter hospital stays and reduced use of intensive care, potentially offsetting the added cost of adopting these tools.

With a relatively small investment to extend the NTAP period, CMS can effectively stimulate the adoption of innovative digital health tools in settings where current payment structures provide limited financial incentives. This extension will support early market entry, encourage evidence generation, and ultimately promote sustained innovation that improves patient care.

Conclusion

CMS plays a critical role in shaping the digital health ecosystem for Medicare beneficiaries. We encourage the agency to lead in building reimbursement frameworks that promote innovation and improve access to digital health solutions.

Thank you for the opportunity to provide comments. We welcome the opportunity to engage further or participate in future rulemaking and demonstration initiatives.

Sincerely,

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